



Fiona Stanley Fremantle Hospitals Group

Policy and Procedure

Kara Maar Duress Response

Reference #: FSFH-MENH-POL-0030

Scope

Site	Service/Department/Unit	Disciplines
Fremantle Hospital - Cockburn Health and Community Centre	Kara Maar Specialist Eating Disorder Service	Nursing, Medical, Allied Health, Peer Workers

1. Introduction

Kara Maar endeavours to meet its Workplace Health and Safety responsibilities to provide a safe work environment and protect staff from harm by providing duress alarms wherever they have been deemed necessary. This document outlines the procedure for responding to a duress alarm when activated to maximise the safety of clients, carers, families, and staff.

2. Terminology

Kara Maar	The Specialist Eating Disorder Service for South Metropolitan Health Service (SMHS) is a community based mental health service located at Cockburn Health and Community Centre. It provides care coordination, clinical monitoring, and multidisciplinary therapeutic interventions to people aged 16 years and over in the SMHS catchment.
Duress alarm	Device used to alert other staff including security when an individual is in need of immediate help. Duress alarms may be wall mounted or provided as a pendant to an individual staff member. Fixed duress alarm means a hard-wired alarm system with duress buttons strategically placed throughout the building.

3. Policy

Staff members must not engage an aggressive person where the level of violence exceeds their capacity to manage the risk.

Staff members must not attempt restraint in the community setting under any circumstances.

- Orientation to duress alarms and the Kara Maar duress response procedures will be included in new staff orientation.
- The Kara Maar Team Leader will ensure that all staff members participate in duress response drills annually.
- Staff are to activate a duress alarm in a situation where they feel that their personal safety or the safety of another is threatened.

3.1. Risk Assessment of New/Unknown Clients

- A risk assessment will be undertaken via review of all available historical and current information (including PSOLIS alerts) prior to the initial assessment interview.
- All initial assessment interviews are to be carried out by two staff members.
- Where a potential risk of aggression is identified, a safety plan must be established in consultation with the multidisciplinary team (MDT).
- Where there is a risk of aggression and it is assessed that the risk may not be contained on the premises at Kara Maar, an alternative site is to be identified or phone/telehealth considered.

4. On site procedure

4.1. General considerations

- Kara Maar is located on the first floor of the Cockburn Health and Community Centre building. It has been assessed as a low-risk area for aggressive incidents and several controls are in place including:
 - Fixed duress alarms are located on the wall of each of the four Kara Maar Consult Rooms (rooms 1 to 4), the Family Interview Room, the Group Room, the Sensory Room, 1 in the Treatment Room, 2 in the Kitchen/Dining area, 2 in the passage, 1 in the Breakout area and 1 in the reception area.
 - Clients are unable to access the clinical and staff areas of the building without security swipe.
 - If a client is already in the building, they are unable to access staff areas without security swipe.

4.2. Maintenance and Testing

- Duress alarms and panel will be serviced every 6 months by a licensed technician and a record of this documented in the Evidence Folder for Kara Maar.
- Fixed duress alarms are to be checked three monthly by staff and a record of this is to be documented in the Evidence Folder for Kara Maar.
- Any duress alarm found to be faulty is to be reported immediately to the Team Leader and clinicians and arrangements made for repairs. The room should be deemed unsuitable for clinical use until the duress alarm is operational.

4.3. Client Interview/Appointment

- Administration staff must be informed when a client interview is taking place at Kara Maar.
- Clinicians external to Kara Maar must not see moderate or high-risk clients at Kara Maar without prior approval of Kara Maar Team Leader or delegate.
- There should be a minimum of two staff onsite whilst a client interview takes place (three staff with a new client). One of the staff members can be administrative staff.
- All clients are escorted to the relevant clinical area by a member of staff/clinician.
- Clinician/s will position themselves in the Consult Room where they have unrestricted access to the duress alarm, and to egress the interview room where it is available (rooms 1 and 3).

4.4. Duress activation

- The staff member, who has activated the duress, may withdraw from the situation if appropriate.
- A blue light on the corridor ceiling outside of the room where the duress has been activated will light up/flash and a loud alarm sound is activated.
- Each staff office and the reception have a control panel and the area where the duress has been activated will light up.
- All staff on the premises will proceed quickly and calmly to the area outside of the room where the duress has been activated and first on the scene will look through the door and communicate the situation.

4.5. Duress response

- The most senior clinical staff member present will provide the leadership role and assess the situation. Where senior clinical staff members are at equal levels, the first senior clinician to attend will take on the leadership role. The leader will direct interventions to contain or prevent escalation of the situation. They may also delegate staff to:

- Liaise with emergency personnel (Police, Ambulance etc.).
- Clear the waiting area and direct all persons to a safe space, as required.
- Intervene, where indicated, without physical intervention, using de-escalation and breakaway techniques as required.
- Administer basic first aid where indicated and safe to do so (staff trained in first aid should provide this response).
- Coordinate communication with other staff in the building and those likely to return/enter building.
- Cancellation of staff the duress alarm will be directed by the response leader / emergency services.

5. Post Incident

- The Team Leader must be informed (if not already aware).
- Team Leader convenes a meeting of all staff involved in the incident to discuss response, staff involvement and outcome.
- Where indicated, the Team Leader will ensure that clinical incident reporting is completed in line with Clinical Incident Management Policy (i.e. harm to a client).
- Where indicated, the Team Leader will ensure that appropriate WH&S reporting is completed (i.e. injury to staff or staff risk issue/hazard identified) using the Combined Hazard or Incident Reporting (CHoIR) system.
- The event should be recorded within the patient medical record (and alert entered on webPSOLIS).
- Team leader to inform Program Manager or delegate of the incident.
- Formal debriefing to be arranged as required by the Team Leader.

6. Compliance Monitoring

The Kara Maar Team Leader will monitor compliance with:

- number of staff trained in API
- percentage of staff participating in annual duress response drills

7. Related Policy Documents

WA Health:

- [Clinical Incident Management Policy](#)

SMHS:

- [Community/Home Visiting SMHS:131](#)
- [Workplace Aggression & Violence SMHS:127](#)
- [Critical Incident Stress Management](#)

FSFHG

- Clinical Risk assessment and Management FSFH-HW-POL-0017

8. Related Standards

NSQHS Standards:

- Clinical Governance

Chief Psychiatrists Standards for Clinical Care:

- Risk Assessment and Management

9. Authorisation

EXECUTIVE SPONSOR: S5 Service Director					
Version	Date Issued	Compiled/Revised By	Committee/Consumer Group Consulted	Endorsed By	Revision due
1	12/23	Team Leader Kara Maar	S5 SQR Committee	S5 SQR Committee	02/2028
1.1	03/2024	CNS – Clinical Improvement Minor Amendments – No change to review date	Manager - Disaster Preparedness and Management	S5 SQR Committee	02/2028